

Tier 2 Secondary Consult

V0 patient flow — two-page executive brief

Planning artifact—not medical or legal advice. Clinical scope, state coverage, consent language, PHI controls, public claims and the RNA reasoner require named licensed review before launch.

RECOMMENDATION

Launch a narrow, coordinator-led pilot

Start with stable adults who have an established cancer diagnosis, a treating clinician and records held mainly by U.S. providers. Pilot the **post-cancer-center** cohort first: people who already have a formal opinion and need differentiated synthesis, targeted testing questions and a provider-ready decision artifact. The clinical clock starts only after a clinician approves completeness and the record packet locks. Use **Tier 2 Secondary Consult** internally for this project. Granola says “Tier 1A,” while the Google pre-read and Notion assignment say “Tier 2.” Avoid tier numbers publicly until the portfolio taxonomy is resolved.

THE PATIENT EXPERIENCE

Six phases above a ten-stage audited workflow

PHASE	PATIENT EXPERIENCE	WHY THIS DECISION
1 · CHECK FIT	Adult, established diagnosis, location, outpatient stability, decision timing, treating clinician.	Creates a safety/licensure/scope gate before payment or PHI retrieval.
2 · AUTHORIZE + FOCUS	Verify identity; service/telehealth consent; access request/authorization; preferences; up to 3 questions.	Keeps consent specific and the review decision-oriented.
3 · WE GATHER	Named coordinator requests, tracks and reconciles records; upload stays optional.	Reduces patient burden and mirrors mature second-opinion programs.
4 · CONFIRM READINESS	Patient verifies timeline/current plan; clinician approves completeness; packet is versioned and locked.	Prevents analysis on mismatched or moving evidence.
5 · EXPERT REVIEW	Technology organizes source-linked evidence; named clinician reviews basis, uncertainty and recommendations.	Preserves accountability; AI/RNA output is never patient-facing without acceptance.
6 · DISCUSS + ACT	One layered PDF, 45–60 minute consultation, teach-back, secure provider delivery and acknowledgement.	Converts synthesis into shared, executable next actions.

THE DELIVERABLE

One PDF, two reading layers

PAGES 1–2	CLINICAL DETAIL
How to use the report; three plain-language answers; agreements/differences; next action, owner and timing; material limitations.	Questions/goals, timeline, diagnosis/stage, treatment/testing inventory, evidence → interpretation → uncertainty → recommendation, hereditary-risk screen, records reviewed/missing, references, signature and version history.

CONTROL MODEL

Hard gates that make the V0 defensible

GATE	V0 CHOICE	RATIONALE
Retrieval	Coordinator-led; patient upload optional	Lowers burden and matches Dana-Farber/AccessHope and Cleveland Clinic patterns.
Completeness	Clinician-owned approval + data cutoff + packet lock	Makes turnaround honest and preserves one evidence version.
Review scope	No raw imaging/pathology in standard V0	Contains logistics and regulatory/clinical risk; impressions remain usable.
RNA reasoner	Clinician-facing trace; independent review required	FDA 2026 CDS guidance makes intended use and reviewable basis material.
Care ownership	Treating clinician remains responsible	Prevents the report from becoming self-directed treatment.
Public/PHI separation	Minimal public fit check; authenticated intake only	Reduces tracking exposure and prevents PHI entering marketing systems.
PHI storage	Approved system, BAA, risk analysis, role access, audit/retention controls	A vendor or BAA alone does not create HIPAA compliance; Drive is conditional only.

OWNERSHIP + PILOT SERVICE LEVELS

One named owner at every handoff

WORKSTREAM	ACCOUNTABLE LEAD	PILOT OUTPUT / TARGET
Patient flow + records permission	Eric + Dan	V0 flow; consent/access matrix; 1-day eligibility contact
Patient/provider PDF	Scott → Jake QA → Dan	Layered template; signed report in 7–10 business days after packet lock
Data + RNA reasoner	Farhan; clinical reviewer accountable	Data dictionary; intended use; source-linked, accepted output
Records + provider flow	Mohit + Jake	Requests within 1 day of authorization; updates every 2 business days; secure handoff
PHI architecture	Security/IT + Dan	Approved system, BAAs, access/audit/retention/incident controls

FOUR-WEEK PATH

Tabletop before any real patient

- **Week 1:** lock scope, taxonomy, consent/access matrix, PDF outline, data dictionary, provider channels and PHI workspace.
- **Week 2:** tabletop both cohorts plus wrong-patient fax, missing pathology, imminent treatment and urgent finding.
- **Week 3:** synthetic dry run from intake through closed-loop handoff; test restore and incident response.
- **Week 4:** enroll 1–2 narrow-fit patients only after clinical, legal/privacy, security and regulatory gates are signed.

IMMEDIATE DECISIONS

Dan: entity/HIPAA role, licensure, consents, report release and urgency policy. Jake/medical lead: clinical completeness and partial-review rules. Security: production PHI system. Farhan + regulatory: RNA reasoner intended use. Team: price, refund, public timeline and whether provider delivery is mandatory.

Evidence base: August 4 Google pre-read, Granola sync and Notion task; NCI/AHRQ; Dana-Farber/AccessHope, Cleveland Clinic, MSK, HHS/ONC/NIST, FDA 2026 CDS, FTC digital-health guidance and peer-reviewed oncology evidence. Full report includes linked sources and decision rationale.